

Before You Try Peptides, Watch This

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Executive Summary

Brandon Carter interviews functional medicine practitioner Amanda Souks on peptides for performance, fat loss, healing, and longevity — stressing that proper testing (blood work, stool, genetics) must come first, and that peptides only amplify a body already primed for results.

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Intro: Brandon's One-Month Peptide Stack 0:00

- Brandon has been experimenting with peptides for about one month, framing it as optimization — not necessity
 - "It's not about needing, it was about optimization — how good can I be?"
- His current stack includes:
 - Tirzepatide (GLP-1/GIP agonist)
 - BPC-157
 - Tesamorelin
 - MOTS-c

- Caveat: taking multiple peptides simultaneously is acknowledged as a poor experimental design
 - Hard to isolate which peptide is producing which effect
- Most noticeable result: Tirzepatide made his cut dramatically easier
 - On a protein-sparing modified fast (~1,000–1,400 calories/day, 300g+ protein)
 - Walking ~40,000 steps/day during the protocol
 - Main challenge was eating *enough* protein — hunger was nearly absent
 - Had to drink multiple protein shakes to hit macros by 8 PM
 - Normally this protocol is extremely difficult; this time it felt easy

Peptides Amplify Your Environment — Not Fix It [1:53](#)

- Amanda's core framework: "Peptides only amplify the environment that they enter into"
 - They can take you from good → great → excellent
 - They work because the body is already primed for them
- If someone is struggling and not getting results, their body may simply not be ready
 - Underlying issues (gut dysfunction, hormonal imbalances, toxin overload) can block peptide effectiveness
- Peptides are not a magic bullet
 - "If you're not putting in the work, if you're not fueling your body, if you're not hydrating, if you're not moving your body — you're wasting money"

Why Blood Work and Root-Cause Testing Must Come First [2:14](#)

- Amanda's strong recommendation: get comprehensive blood work *before* starting any peptide protocol
 - Especially check IGF-1 levels — negative consequences from certain peptides occur when biomarkers aren't in the right place
- Brandon's current cadence: blood work every 1–3 months (has been on TRT for ~5 years, since just before age 39)
- The business analogy: "Why would we run our businesses but not run our bodies the same exact way — with data, with systems and structure?"
- Root-cause lens: symptoms are signals/gifts telling you something needs to be fixed
 - Is there gut dysfunction?
 - Is there toxin overload?
 - Are hormones or adrenals off?
- Three people with the same symptom (e.g., waking at night) may each need a *different* peptide — one size does not fit all
- Lab ranges on standard blood work are based on 95% of the population
 - That population skews sick, so "normal" ≠ optimal

- Example: Brandon's testosterone was ~400–500 ("normal" range) but he had naturally tested at ~1,200 in his 20s — felt terrible at the lower number

Gut Health: Stool Testing, Cortisol, and the Gut-Brain Connection 5:03

- Brandon's honest skepticism: gut health talk often felt like an excuse to sell probiotics
- Amanda's rebuttal: probiotics can actually make things *worse* — they can add fuel to the fire
 - Most probiotic marketing is a quick fix, not a solution
- The real tool: a **stool test** — 30-page report shipped to your home
 - Reveals: pathogens, parasites, bacterial overgrowth
 - Results back in ~3 weeks
 - Process: place a provided liner over the toilet, scoop sample into a vial, mail to lab
- Why the gut matters:
 - Gut lining is only **one cell layer thick**
 - High performers chronically elevate cortisol (stress hormone)
 - Excess cortisol over time breaks down the gut lining
 - Result: toxins re-enter the bloodstream, cross the blood-brain barrier → downstream autoimmune conditions, mental health challenges
 - **70% of the immune system resides in the gut**
- Additional testing Amanda recommends alongside stool test:
 - Heavy metals testing
 - Mold/mycotoxin testing
 - Genetics panel (see genetics section)

Amanda's Health Collapse and How Peptides Saved Her 7:07

- Amanda was a Dallas Cowboys cheerleader finalist; two months after that, her health collapsed
- Went from 8–9 hours of productive work per day down to 2 hours
 - Gained **35 lbs** seemingly overnight
 - Body fat jumped from ~15% (very lean for a woman) up to **28.5%**
 - Couldn't think clearly, couldn't sleep through the night, lost energy entirely
 - Estrogen levels were post-menopausal despite being in her mid-20s
- Saw **five or six doctors** with no answers
- Ran a stool test as a last resort → discovered a bacterial pathogen: **C. diff (C. difficile)**
 - Typically seen in people over 65 in hospital settings on antibiotics
 - Finding it in a healthy woman in her mid-20s was remarkable
- Spent ~18 months healing her gut through trial and error

- After gut was healed, discovered **black mold** (toxic mold from building exposure, common in Florida/Miami) in her body
 - Met a doctor who had healed his wife from mold using peptides
 - Peptide used: **VIP (Vasoactive Intestinal Peptide)**
 - Took **~3 months** to clear the mold
- During VIP protocol, body weight didn't budge for **77 days**
 - On day 77: dropped **10 lbs overnight** — the body had been holding massive water weight and inflammation
 - Weight stayed off the next day
 - Eventually brought body fat from 28.5% back down to 15–17%

Epitalon and Tesamorelin: Sleep Reset and Circadian Rhythm 8:01

- First peptide Amanda was introduced to: **Epitalon (Epithalon)**
 - Known for longevity — works by extending DNA (telomere-related mechanism)
 - Key function: **resets the pineal gland**, which resets the circadian rhythm (24-hour sleep cycle)
 - Protocol: **10-day reset**
- Average American gets only **35–45% restorative sleep** (deep sleep + REM)
 - Deep sleep = body recovery; REM sleep = brain recovery
- Amanda's personal result:
 - Started at **48% restorative sleep** (already above average)
 - After just **3 days** into the 10-day reset: jumped to **60%** restorative sleep
 - Has stayed elevated since
- Brandon's sleep experience on Epitalon + Tesamorelin:
 - Has bipolar II — experiences mild manic episodes where sleep suffers
 - Was mid-episode when he started
 - Sleep became the best it had been in **10–15 years**
 - "Soon as my head hits the pillow, I'm asleep" — describes it as time travel
 - Tracks with Oura Ring: data confirmed the improvement
- Amanda attributes the sleep benefit primarily to **Tesamorelin** (growth hormone releasing hormone analog)

BPC-157 and TB-500: The Wolverine Stack for Injury Repair 13:33

- **BPC-157**: performs cellular repair work
- **TB-500**: stimulates the body's own stem cell production
 - Together they are highly synergistic for injury healing
- Important warning on blends: do NOT buy pre-blended vials of BPC-157 + TB-500

- The two peptides have different pH balances
- Mixing them in one vial can **denature** (destroy) the integrity of the peptides
- Pre-blends are also cheaper — "that should be a sign"
- Buy them separately and administer separately
- Amanda's personal injury story:
 - Had a stubborn ankle injury with an audition one month away
 - Was doing PT at the same facility where Dak Prescott does offseason PT
 - Nothing was fixing the ankle
 - Started BPC-157 + TB-500 injury protocol
 - Was back on the treadmill doing **sprints within 10 days**
- Brandon's experience:
 - Had chronic aches in traps and bicep for nearly a decade
 - Also got exosomes (IV, systemic) — helped a little but not dramatically
 - After BPC-157 + TB-500: "I have to look for the pain in my bicep and traps now"
 - Acknowledges possible placebo but notes the evidence feels real
- Legality note: BPC-157 and TB-500 are **banned in professional sports**; they can technically be detected on tests
 - Brandon's take: "Life is not a drug-tested sport — take advantage"

AOD-9604 and Tesamorelin: Fat Loss Stack [10:34](#)

- Amanda's preferred fat-loss peptide stack:
 - **Tesamorelin** (growth hormone releasing hormone) — stacked with
 - **AOD-9604** (Anti-Obesity Drug) — specifically targets fat
 - Despite the name, it is a peptide fragment of human growth hormone
- These are the "aesthetic" peptides Amanda uses for herself during body recomposition phases
- Reminder: peptides are not magic without the foundation
 - Must be fueling properly, hydrating, and moving

MOTS-c: Stable Energy and HRV [3:22](#)

- **MOTS-c**: mitochondria-derived peptide
 - Known for endurance athletes — helps cardiovascular performance
 - Also targets fat effectively
 - Amanda: helps with heart rate variability (HRV)
- Brandon's experience on MOTS-c during his most intensive diet protocol:
 - Normally needs **~1g of caffeine** to get through the 40,000-steps/day + crash-diet protocol
 - With MOTS-c: didn't feel he needed as much caffeine

- Energy felt like a "light dimmer turned up and staying" — not a jolt like pre-workout
- "Stable but high above baseline" — like a thermostat raised permanently
- Amanda: "Almost like a light dimmer that goes up and then just stays" — confirms this is normal *if the body is ready*
- Brandon's HRV (Oura Ring): consistently in the **20s** (weekly average ~20, highest ever ~24)
 - Amanda flagged this as low and something to address
 - HRV plummeting was her first leading indicator when she got sick
 - Also noted: Brandon's years of staying extremely lean + intense training may have taxed his endocrine system and could be connected to low HRV
 - Amanda works with an Ironman athlete whose HRV is in the 30s with the same complaint

Genetics Testing: 160+ Genes for Personalized Peptide Selection 16:38

- Amanda offers a **genetics panel** that tests **160+ genes**, broken into **36 categories**
 - Genes are clustered and ranked by level of impact on your body
 - Delivers a **30-page report**
 - Process: a kit is shipped to you → mouth swab (inside cheek, 30 seconds) → mail back → results in **4 weeks**
- What it reveals:
 - Prone to oxidative stress? → Peptide **SS-31** can reduce that marker
 - Brain/memory health challenges? → Specific peptides exist for that
 - Inflammation impact level → dictates how critical sleep and nutrition are for you
 - Workout type suitability: are you a power athlete or endurance athlete?
 - How much recovery you need
- Amanda feeds the results into her proprietary algorithm:
 - Combines genetics data + blood work + other testing
 - Outputs recommendations across nutrition, lifestyle, supplementation, and peptide selection
- This eliminates guessing about what your body needs
- Example condition: **KPV** (tripeptide — three amino acids)
 - Helps with inflammation and gut health
 - Amanda cautions: a peptide won't fix inflammation overnight if foundations (gut, hormones, adrenals) aren't in place

Tirzepatide vs. Retatrutide, and Peptide Safety Considerations 12:08

- Brandon chose **Tirzepatide** over **Retatrutide** for two reasons:
 - Tirzepatide is **legal** (FDA-approved); Retatrutide is not yet

- Retatrutide hits a **third receptor** (GIP + GLP-1 + glucagon), which can raise heart rate and make you feel more stimulated — not desirable for him
- Also heard Retatrutide may not be ideal on a **low-carb/keto** diet (unverified)
- Brandon has been **keto for 11 years**, staying under 50g net carbs/day
- Gets Tirzepatide prescribed through a doctor
- Who should NOT take certain peptides:
 - People with a **history of cancer** — certain peptides are contraindicated
 - People with **gut dysfunction** — can make things worse
 - Bodybuilders who have pushed their gut to extremes are particularly at risk
- Amanda's nuance: always look at past health history before selecting a protocol
- Peptides used therapeutically (not just aesthetics/performance):
 - **VIP**: cleared black mold toxicity
 - **Epitalon**: longevity and circadian reset
 - **KPV**: inflammation and gut
 - **SS-31**: oxidative stress reduction

Healthspan vs. Lifespan: The 14-Year Gap 19:13

- Striking statistic Amanda shares:
 - Average US **lifespan**: **78 years**
 - Average US **healthspan**: **64 years**
 - Healthspan = years free of chronic conditions (defined as lasting 1+ year)
 - Gap = **14 years** of being alive but not truly living
- Most chronic disease didn't happen overnight — it built over 5, 10, 15, 20 years
 - Preventive testing catches problems before the warning light comes on
 - Analogy: planes don't fall from the sky because engines are checked after every flight; cars alert you via dashboard lights before failure
- Brandon's approach: gets EKGs and full-body MRIs when nothing is wrong — purely preventive

"Just because something is common doesn't make it normal."

- The lab range problem: standard blood work flags anything within 95th percentile as "normal" — but that population base skews toward sick people, not optimal ones

Raising Testosterone Naturally: Micronutrients, Gut, Toxins 22:09

- Amanda's approach: hormones are often a **secondary signal**, not the root problem
 - Fix the root first: micronutrients → gut → toxin load → hormones often self-correct

- Key micronutrient: **Vitamin D** — most people are deficient
 - Especially Black and Latino people: skin evolved near the equator for high sun exposure; less absorption in modern environments
- Amanda's 3-lever framework for natural hormone optimization:
 - Fix micronutrients
 - Fix gut health
 - Reduce toxin load
 - Once all three are addressed, sex hormones AND thyroid hormones tend to normalize naturally
- Case study: client in Park City, worked with for 6 months
 - Month 1 to Month 6: **doubled her testosterone levels naturally**
- Brandon's TRT story:
 - Testosterone was ~400–500 (low end of "normal")
 - Had naturally tested at ~1,200 in his 20s
 - Tried eating carbs again (thought keto was suppressing T) — didn't help; studies showed keto can *raise* testosterone
 - Watched Biden and Trump campaign and thought: "These fat old men are running circles around me — I need to fix this"
 - Went on TRT; has felt great ever since
 - Hypothesis for his low T: **20+ years of staying very lean** may have taxed his endocrine system
- Amanda's line: "400 testosterone is so low. No man should have testosterone anywhere near 400."
- When to refer for HRT: Amanda will refer out for TRT when natural methods aren't enough — acknowledges the value, but notes once you start TRT you generally stay on it



Tracking Devices: Oura Ring vs. Whoop [22:27](#)

- Brandon uses **Oura Ring** (Gen 4, size 11, brushed finish to hide scratches)
 - Wears it 24/7 including lifting
 - Uses it to track calories (acknowledges not 100% accurate but good barometer)
 - Aesthetic preference: blends with watches and suits; Whoop looks out of place with formalwear
 - Concern: newer Oura rings are thinner and look "girly" to him — may need to find an alternative when current one dies
 - Has gone through four Oura rings total
- Amanda uses **Whoop** (3+ years of data)
 - More geared toward performance optimization
 - Now includes women's cycle tracking and its impact on HRV, sleep, and recovery
 - Recommends it to clients
- Consensus: both devices provide essentially the same core data

- Choice comes down to ring vs. wrist band preference
- Even an **Apple Watch** is better than nothing — creates awareness around sleep and recovery habits
- Brandon also uses **Claude AI** (instead of MyFitnessPal) to track nutrition
 - Syncs with Apple Health
 - Has a custom prompt that checks calories burned and tells him his current deficit in real time

Resources: Peptide Playbook and Amanda's Custom Blueprint 26:18

- **Free Peptide Playbook** — Amanda's offer for Brandon's audience:
 - DM the word "**KingKeto**" (one word) to Amanda on Instagram
 - Instagram: **@AmandaSouks** (A-M-A-N-D-A S-O-U-K-S)
 - Playbook covers: which peptides are best for which goals, how to handle adverse reactions, which peptides to avoid in certain conditions
- **Precision Pathway / Custom Blueprint** — also available via DM:
 - Amanda's first-call process for new clients
 - Reviews complete health history, goals, and symptoms
 - Leads to a personalized protocol combining all testing data
- **Peptides source** Brandon uses: **godmodesciences.com**
- Amanda's full testing protocol for new clients:
 - Blood work (hormones, IGF-1, micronutrients, vitamin D)
 - Stool test (30-page gut report, ~3-week turnaround)
 - Genetics panel (160+ genes, mouth swab, ~4-week turnaround)
 - Optional: heavy metals, mold/mycotoxin testing
 - All results fed into algorithm → custom nutrition, lifestyle, supplementation, and peptide plan